

NURSING VALUE ASSESSMENT

Deaconess - Gateway ICU

56 beds · 148 nurse FTEs · Inpatient Nursing

PREPARED BY

Christina Searle · 4/28/2026 · April 28, 2026



Where Nursing Value Actually Lives

Ambient documentation creates value across four distinct buckets. For nursing, three of them carry real dollars — and one of them, honestly, doesn't apply. We separate them because the strategic implications of each are different.

Nurses don't bill. So where does the value live?

Nursing documentation creates value through quality (preventable harm and bundle compliance), workforce (retention, agency, and overtime), and capacity (more time at the bedside). Revenue — the fourth bucket — is where nursing intentionally doesn't play.

CAPACITY

Tracked

Reclaimed documentation time goes back to bedside care. We track the ratio of bedside-to-charting time post-deployment as a leading indicator that the model is working.

WORKFORCE

\$389K

Documentation burden is among the top drivers of nurse turnover. The hypothesis we model: reducing end-of-shift charting moves retention, the agency spend that follows every vacancy, and the overtime budget — three distinct labor lines that all share one root cause.

QUALITY

\$0 (potential)

Preventable harm events — HAPIs, falls, CAUTI, CLABSI — and sepsis bundle compliance are sensitive to documentation timing. The hypothesis we model: real-time flowsheet capture surfaces the visibility for earlier intervention, with each unit's actual outcome shaped by clinical practice.

REVENUE

Not Applicable

Nursing documentation doesn't generate billing revenue. The value lives in labor economics and care quality — which is where we've modeled it.

WHAT YOUR CHOICES REVEAL

Your model combines overtime reduction with retention savings. OT reduction is grounded in your current overtime spend; retention reflects the burnout-related turnover that documentation burden accelerates. Together they form a defensible staffing ROI.

Documentation burden is the reason nurses leave. And the reason they stay late.

Three labor lines — turnover, agency premium, and overtime — and one leading-indicator capacity line, all driven by the same root cause: how long it takes to finish charting at the end of a shift.

WORKFORCE

RN RETENTION

\$112K

Documentation burden drives burnout drives turnover. Reducing burden helps retain experienced nurses who would otherwise leave.

NURSE FTES	148
ANNUAL TURNOVER RATE	18%
BURNOUT-RELATED SHARE	40%
DOCUMENTATION IMPACT ON BURNOUT TURNOVER	15%
REPLACEMENT COST PER NURSE	\$70,000
= RETENTION SAVINGS	\$112K

Source: NSI Nursing Solutions 2024 turnover benchmark.

OVERTIME REDUCTION

\$277K

When nurses spend less time documenting at end of shift, OT hours decrease. The most directly measurable line in the payroll budget.

NURSE FTES	148
OT HRS PER NURSE PER WEEK	1.6 hrs
DOCUMENTATION-DRIVEN OT REDUCTION	30%
OT HOURLY RATE	\$75/hr
WEEKS PER YEAR	52
= OT SAVINGS	\$277K

WORKFORCE SUBTOTAL \$389K

Capacity for nursing isn't a billable line — it's a leading indicator. When nurses finish charting on shift, the next hour goes to direct patient care instead of catch-up documentation.

PATIENT DAYS/YR	HRS RETURNED/YR	MIN SAVED/SHIFT	STAFFED BEDS
18,396	3,848	20	56

Real-time documentation is the visibility layer that makes early intervention possible.

Preventable harm events don't happen because nurses don't care — they happen when risk signals are missed or delayed. The hypothesis we model: real-time flowsheet capture surfaces those signals while there's still time to act, with the actual outcome determined by clinical practice on each unit.

The values below are potential — they require clinical practice change alongside documentation improvement. Documentation creates the visibility; the care team creates the outcome. Validate baseline rates with your infection-control and quality teams before presenting.

Nursing documentation doesn't generate billing revenue.

That's by design — and it's worth saying directly.

Every other care setting in this model has a revenue optimization story — wRVU lift, DRG accuracy, denial prevention. Nursing doesn't. Nurses don't bill. Their documentation drives care quality and labor economics, not the revenue cycle.

We've modeled the value where it actually lives: in the QUALITY bucket (preventable harm events, sepsis bundle compliance) and the WORKFORCE bucket (retention, agency cost avoidance, overtime reduction). Presenting a nursing revenue story that doesn't exist would undermine the credibility of the model that does.

WHAT THIS MEANS FOR YOUR PRESENTATION

When a CFO asks where the nursing revenue story is, the answer is: there isn't one — and that's the point. The ROI case for nursing ambient AI is a labor economics and care quality story. It stands on its own. You don't need to manufacture a billing narrative to make it compelling.

THE INDIRECT CONNECTION

Complete nursing documentation does create downstream organizational value: HCAHPS performance affects Value-Based Purchasing scores. HAC scores affect Medicare payment rates. Sepsis SEP-1 compliance affects public reporting and payer relationships. These are real — they're just not direct billing revenue, and we've modeled them as quality outcomes rather than revenue line items, which is the more defensible framing.

THE INVESTMENT CASE

Infrastructure, Not Expense.

The investment is \$91K annually – \$136/bed/month at 56 staffed beds. Years 2–3 assume 15% growth as adoption matures and documentation habits stabilize across the unit.

PERIOD	VALUE	INVESTMENT	NET VALUE	CUMULATIVE
Year 1	\$389K	\$91K	\$298K	\$298K
Year 2	\$447K	\$91K	\$356K	\$653K
Year 3	\$514K	\$91K	\$423K	\$1.08M

By Year 3, the program is generating 3.9× cumulative net for every \$1 invested – while your nursing staff spends more time at the bedside and less time charting after their shift.

AT SCALE

CURRENT MODEL

\$389K/yr

56 beds · 50% adoption

Per bed: \$5,313/yr

AT FULL SCALE

\$1.56M/yr

224 beds · 80% adoption

Per bed: \$5,313/yr

Illustrative scaling, not a hard projection.

KEY METRICS TO TRACK

1. Documentation time per shift (target: –40%)
2. Nurse satisfaction / burnout score (target: +15 pts)
3. Overtime hours per FTE per week (target: –25%)

YOUR ASSESSMENT SUMMARY

56 staffed beds · 148 nurse FTEs.

\$298K projected net value

\$5,313/bed per year

VALUE SUMMARY

QUALITY	\$0 (potential)
HAPI Prevention	—
Falls Prevention	—
CAUTI Prevention	—
CLABSI Prevention	—
Sepsis Bundle Compliance	—
HAC Penalty Exposure	Risk display only
HCAHPS / Patient Experience	—
Medication Error Reduction	—
WORKFORCE	\$389K
RN Retention	\$112K
Agency & Travel Nurse Reduction	—
Overtime Reduction	\$277K
CAPACITY	Tracked
Bedside / Direct Care Time	Tracked
REVENUE	Not Applicable
Nursing documentation does not generate billing revenue.	
NET ANNUAL VALUE (HARD)	\$298K

METHODOLOGY

- Retention: 18% annual turnover, 40% burnout-related. 15% impact scenario. \$70,000 replacement cost. Source: NSI Nursing Solutions 2024.
- OT Reduction: 1.6 OT hrs/nurse/week at 30% reduction. \$75/hour.
- HAC Penalty: 1% of Medicare revenue if in bottom quartile. Risk display only — not included in ROI total.
- Revenue: Not applicable. Nursing documentation does not generate billing revenue.

This assessment is for planning purposes. Hard value projections are based on user-provided staffing inputs. Potential value uses published clinical rates with documentation-attributable prevention rates that reflect the indirect causal chain. Validate with your organization's data post-implementation.